

SURGERY FOR ISCHAEMIC HEART DISEASE (Revision Table)

Bold = high-yield. Dr Shkar Saeed.

PATHOPHYSIOLOGY, DIAGNOSIS & MEDICAL

Topic	Key points
Pathophysiology	Atherothrombotic coronary blockage → stable angina ; plaque rupture + thrombosis → ACS (unstable angina/NSTEMI/STEMI)
Diagnosis	Clinical, troponin, ECG, treadmill, echo, CT angio , coronary angio
Medical	Aspirin , beta-blocker , nitrate , CCB , statin (plaque stabilization) , ACE inhibitor , clopidogrel
STEMI	PCI (90-min door-to-balloon) or thrombolytic; "time is myocardium"
ACS	Aspirin + heparin; clopidogrel; prasugrel stop 7 days pre-op ; GP IIb/IIIa (tirofiban) if ongoing ischaemia
PCI	BMS vs DES

SURGERY (CABG)

Topic	Key points
Benefit	Relieves angina, delays infarction, improves survival vs medical/PCI
Clinical indications	Class III/IV angina, ischaemic pulmonary oedema, failed PCI, major vascular surgery
Anatomic indications	Left main >50% ; 3VD + EF <50% ; 3VD EF>50% + inducible ischaemia ; lesions not PCI-amenable
Other	>50% branch stenosis + valve surgery, MI complications, coronary anomalies
Conduits	Venous vs arterial (LIMA superior)
Procedures	On-pump CABG, OPCAB (off-pump) , MIDCAB, TECAB, robotic, laser TMR
Post-op	Beta-blocker, aspirin, statin, ACE inhibitor
MI complications	Septal rupture , free wall rupture , ischaemic MR , LV aneurysm